Frog Logic

Tourette's & Tics

A POND GUIDE

Almost everything most people think they know about Tourette's comes from one rare symptom being used as a punchline for forty years.

In plain words

Tourette Syndrome involves both motor and vocal tics — sudden, repeated movements or sounds — present for over a year. Tics aren't voluntary, though many people can hold them briefly at a cost. Involuntary swearing is real but affects only a small minority, roughly one in ten.

What it can feel like from the inside

- A building pressure before a tic that only doing it will release.
- Being able to suppress in public, then a much bigger wave the moment you're home.
- Tics worsening when you're stressed, tired, excited — or being watched.
- People assuming you're doing it deliberately, or for attention.
- Exhaustion from managing something that never stops.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"Tourette's is swearing."	Coprolalia affects around 10%. Most tics are blinking, throat-clearing, shrugging, small sounds.
"They could stop if they wanted."	Suppression is possible briefly, like holding a sneeze, and just as unsustainable.
"They're doing it for attention."	Almost everyone with tics would very much prefer not to be noticed.
"It's just nerves."	It's neurological. Stress worsens it; stress didn't cause it.
"It's funny."	It's frequently painful, and it isn't a bit.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Humour, often sharp, and well-earned.

Resilience from managing something visible in public every day.

Genuine compassion for anyone who stands out.

Creativity — tics travel with a lot of creative minds.

Honesty about difficulty, having never had the option of hiding it.

What actually helps

- ☐ Don't mention or count the tics
- ☐ Carry on the conversation normally
- ☐ Never mimic, even affectionately
- ☐ Don't tell them to relax
- ☐ Offer a quieter space if it helps
- ☐ Let them leave and come back

If someone tells you this is them

HELPFUL

Nothing at all — just carry on.

"Want to sit somewhere quieter?"

"Doesn't bother me."

LESS HELPFUL

"Just relax."

"You did it again."

"Say something rude then!"

Not a joke, not a choice, and rarely what television told you it was.

A friendly explainer written for the community — not a diagnostic tool and not medical advice. If you want an assessment, your GP is the place to start. Nobody experiences this identically, so take what fits and leave the rest.

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